SACTCHECK TREATMENT ASSESSMENT

Bevacizumab + FOLFIRI

NCCP 00449 | 6a
SACTCheck v0.83.3

TUMOUR GROUP	INDICATION	COURSE
Gastrointestinal	**Treatment of adult patients with metastatic carcinoma of the colon or rectum.**	**Course metadata not confirmed**

TREATMENT CONTEXT	ASSESSMENT ID	ASSESSMENT TIME
Not entered	**JSON-DEMO-001**	**16/10/2026, 22:20**

ENCODED CRITERIA RESULT

1 encoded criterion requires clinical review

Review the highlighted protocol criteria and the current NCCP source before the final treatment decision.

ENTERED VALUES AND ENCODED CRITERIA

Standard assessment

DOMAIN	ENTERED VALUE	ENCODED CRITERION	RESULT
ANC (x10^9/L)	1.5	>= 1.5	**Dose-modification pathway**
Lowest ANC during delayed week(s) (x10^9/L)	0.49	< 0.5	**Dose-modification pathway**

COMPONENT-SPECIFIC PROTOCOL ACTIONS

Protocol-level guidance; verify prescribed dose/current level

Irinotecan: Dose level -2 - 120 mg/m^2
Reduce irinotecan and both fluorouracil components by two dose levels. page 4 | Table 2

Bolus 5-FU: Dose level -2 - 260 mg/m^2
Reduce irinotecan and both fluorouracil components by two dose levels. page 4 | Table 2

Infusional 5-FU: Dose level -2 - 1500 mg/m^2
Reduce irinotecan and both fluorouracil components by two dose levels. page 4 | Table 2

UNASSESSED DOMAINS

Known hypersensitivity to regimen components, Pregnant, Breastfeeding, Chronic bowel disease and/or bowel obstruction, DPD status, Assessment type, Baseline greater than 3 loose bowel movements per day (without stoma), Known homozygous UGT1A1*28 genotype + 22 more

Blank domains were not assumed normal.

CLINICIAN DECISION

Not documented
Not documented

CLINICAL DECISION SUPPORT - NOT TREATMENT CLEARANCE

SACTCheck applies encoded NCCP protocol criteria to clinician-entered information. This output does not constitute treatment clearance and does not replace complete clinical assessment, the current official NCCP protocol, prescribing information, local policy or professional judgement. The responsible oncology clinician retains responsibility for the final treatment decision.